

Article Summary and Opinion

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Name of Article	Psychosis and Mass Shootings: A Systematic Examination Using Publicly Available Data
Name of Journal	Psychology, Public Policy, and Law (American Psychological Association)
Article Authors	Jillian K. Peterson, James A. Densley, Kyle Knapp, Stasia Higgins, and Amanda Jensen
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Summary

Peterson, Densley, Knapp, Higgins, and Jensen take on a claim that gets repeated after nearly every mass shooting in the United States: that serious mental illness is the cause. Rather than asking whether shooters had a diagnosis, which earlier studies had done, they ask a narrower and more useful question. How often did symptoms of psychosis, meaning delusions or hallucinations, actually motivate the shooting, and how do shooters driven by psychosis compare with everyone else?

The study draws on The Violence Project database, a federally funded and publicly available dataset covering 172 perpetrators of 168 mass public shootings from 1966 through 2020. The authors used the Congressional Research Service definition, which requires four or more victims killed with firearms in a public place and excludes shootings tied to gang activity, robbery, or private family disputes. The team coded 166 life-history variables for each shooter from court records, manifestos, interviews, news archives, and other open sources, with every case reviewed by several independent coders. For psychosis specifically, they used a four-point continuum borrowed from earlier research on offenders with mental illness: no role, minor role, moderate role, or major role. A major role meant the shooter was acting on delusions or hallucinations while planning and carrying out the attack and had no other discernible motive.

The headline result is that psychosis played no role in roughly 70 percent of the shootings. It played a minor role in 11 percent, a moderate role in about 9 percent, and a major role in about 10 percent. Put differently, psychosis was the central driver in about one case in

ten and some part of the picture in about three in ten, and that share stayed fairly stable across five decades. At the same time, mental health history in the broad sense was common. Close to 60 percent of shooters had some record of hospitalization, counseling, medication, or a diagnosis, and about a quarter showed evidence of a psychotic disorder, far above the rate in the general population. The authors stress that history and motive are not the same thing. A diagnosis in someone's past does not mean symptoms drove the attack.

When shooters motivated by psychosis were compared with the rest, they looked remarkably alike. There were no meaningful differences in age, race, criminal record, history of violence, substance use, employment, or relationship status. The one statistically significant difference was education, with the psychosis group more likely to have attended college. Their firearm behavior matched as well. Handguns were the most common weapon, most guns were bought legally, and more than half of the firearms used by shooters with a prior psychiatric hospitalization were legal purchases, which the authors read as a sign that the federal prohibition on gun sales to people who have been involuntarily committed is not working as intended.

From these findings the authors argue that psychosis is a poor proxy for dangerousness and that policy should target dangerous situations and behaviors instead. They point to universal background checks, extreme risk protection orders, systems that recover firearms from owners who become prohibited after a purchase, and school and workplace threat assessment teams that operate more like crisis response than discipline. They also note that hate and fame-seeking drive many shootings and support responses such as limiting media use of perpetrators' names.

Opinion

What I appreciate most about this article is that it refuses to give a simple answer in either direction. It would have been easy to report the 70 percent figure and declare mental illness irrelevant, or to report the 60 percent mental health history figure and confirm the popular narrative. Instead the authors separate the two and show that both are true at once, which is a more honest picture than most news coverage offers.

I do think the reliance on public records deserves more weight than the authors give it. Media coverage is uneven, the authors themselves cite evidence that reporters describe White shooters as mentally ill more readily than Black shooters, and some diagnoses were made only after arrest. The psychosis estimate could be off in either direction, and the lack of a

comparison group makes it hard to know how unusual a 27 percent psychotic disorder rate is among violent offenders generally. The authors acknowledge these problems, but they could shape the interpretation more than the discussion admits.

Still, the policy section is where the article earns its place in a law journal. The finding that previously hospitalized shooters legally bought most of their guns describes a concrete failure of existing law, not a problem of public attitudes. That points toward fixing reporting and background check systems, which is a far more achievable goal than trying to predict which of the millions of Americans living with a mental illness might someday become violent.